

DEPARTMENT OF REGULATORY AGENCIES

State Board of Examiners of Nursing Home Administrators

NURSING HOME ADMINISTRATORS RULES AND REGULATIONS

3 CCR 717-1

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

1.1 AUTHORITY

These regulations are adopted pursuant to the authority in sections 12-20-204 and 12-265-107(1)(a), C.R.S., and are intended to be consistent with the requirements of the State Administrative Procedures Act, section 24-4-101 *et seq.* (the "APA"), C.R.S., and the Nursing Home Administrators Practice Act, sections 12-265-101 *et seq.* (the "Practice Act"), C.R.S.

1.2 SCOPE AND PURPOSE

These regulations shall govern the process to become a licensed nursing home administrator and the practice of nursing home administration in Colorado.

1.3 APPLICABILITY

The provisions of these regulations shall be applicable to the practice of nursing home administration in Colorado.

1.4 CHANGE OF NAME AND ADDRESS

This Rule is promulgated pursuant to sections 12-20-204 and 12-265-107(1)(a), C.R.S.

- A. A licensee shall inform the Board in a clear, explicit, and unambiguous written statement of any name or address change within thirty days of the change. The Board will not change the licensee's information without explicit written notification from the licensee. Notification by fax or email is acceptable.
 - 1. The Division of Professions and Occupations maintains one contact address for each licensee, regardless of the number of different professional licenses the licensee may hold.
 - 2. All communication from the Board to a licensee will be to the contact address maintained with the Division of Professions and Occupations.
- B. The Board requires one of the following forms of documentation to change a licensee's name or correct a social security number or individual taxpayer identification number:
 - 1. Marriage license;
 - 2. Divorce decree;
 - 3. Court order; or

4. Documentation from the Internal Revenue Service verifying the licensee's valid individual taxpayer identification number; or
 5. A driver's license or social security card with a second form of identification may be acceptable at the discretion of the Director of Support Services.
- C. Except for letters of admonition, notifications by the Board to a licensee or applicant, required or permitted, under section 12-265-101, *et seq.*, C.R.S., or under section 24-4-101, *et seq.*, C.R.S. (State Administrative Procedure Act), shall be served personally, by first class mail, or electronically to the last address of record provided in writing to the Board and maintained by the Division of Professions and Occupations. Service by mail or electronic mail shall be deemed sufficient and proper upon a licensee or applicant. Letters of admonition must be served by certified mail pursuant to section 12-265-113(3), C.R.S.

1.5 GENERAL LICENSING REQUIREMENTS

This Rule is promulgated pursuant to sections 12-20-204, 12-265-107(1)(a), 12-265-108, 12-265-110, and 12-265-111, C.R.S.

- A. With regard to all licensure applications, except for licensure renewal:
1. All documents required as part of a licensure application must be received within one year of the date of receipt of application;
 2. An application is incomplete until the Board receives all additional information requested or required in order to determine whether to grant or deny the application; and
 3. If all required information is not submitted within the one year period, then the original application materials will be destroyed and the applicant will be required to submit a new application, fee, and all required documentation.
- B. All applicants who have met the education/experience requirements and have otherwise complied with the Rules of the Board must successfully pass a Board approved national and state examination.
- C. The national examination shall consist of those questions constituting an examination formulated by a national association, approved by the Board.
- D. The content areas covered by the state examination may include questions in the following areas:
1. Long-Term Care Facilities: including, but not limited to laws, powers, and duties in accordance with sections 25-1.5-103 and 25-3-101, C.R.S., *et seq.* and standards promulgated by the State Board of Health for licensure of long term care facilities.
 2. Licensure and Building and Fire Safety: including, but not limited to standards promulgated by the State Board of Health for general licensure of hospitals and health facilities, and standards for general building safety; and the Division of Fire Safety in the Office of Preparedness, Security, and Fire Safety within the Department of Public Safety for fire safety for all licensed health facilities.
 3. Laws and Rules Governing Nursing Home Administrators: including, but not limited to laws, powers, and duties in accordance with section 12-265-101, C.R.S., *et seq.*, and these rules and regulations for nursing home administrators promulgated by the Board of Examiners of Nursing Home Administrators.

- E. Applicants shall be informed in writing regarding examination performance.
- F. Applicants seeking licensure pursuant to section 12-265-108, C.R.S., must successfully complete the national and state examinations within one year after approval to take the examination has been granted. If the candidate does not successfully complete the examinations within the time period specified, the candidate shall be required to reapply for licensure and pay all appropriate fees.

1.6 EDUCATION, TRAINING, OR SERVICE GAINED DURING MILITARY SERVICE AND MILITARY SPOUSES

This Rule is promulgated pursuant to sections 12-20-202(4), 12-20-204, and 12-265-107(1)(a), C.R.S.

- A. Education, training, or service gained in military services outlined in section 12-20-202(4), C.R.S., to be accepted and applied towards receiving a license, must be substantially equivalent, as determined by the Board, to the qualifications otherwise applicable at the time of receipt of the application. It is the applicant's responsibility to provide timely and complete evidence for review and consideration. Satisfactory evidence of such education, training, or service will be assessed on a case by case basis.
- B. Regulations of Military Spouses.
 - 1. The spouse of a person who is actively serving in the United States Armed Forces and who is stationed in Colorado in accordance with military orders shall be entitled to a temporary license to practice nursing home administration as set forth in section 12-20-202, C.R.S., subject to the following terms:
 - a. The person is licensed to practice as a nursing home administrator in another state;
 - b. Other than the person's lack of licensure, registration, or certification in Colorado, there is no basis to disqualify the person under Title 12 of the Colorado Revised Statutes; and
 - c. The person consents, as a condition of practicing in Colorado, to be subject to the jurisdiction and disciplinary authority of the Board.
 - 2. Nothing in sections 12-20-301 through 12-20-305, C.R.S., abrogates the requirements of Long Term Care Facilities for licensure as set forth in 6 CCR 1011-1, Chapter V, or for participating in Medicaid or Medicare as set forth in 42 CFR Part 483.

1.7 LICENSURE BY EXAMINATION

This Rule is promulgated pursuant to sections 12-20-204, 12-265-107(1)(a), 12-265-108, 12-265-110, 12-265-111, C.R.S.

- A. Applicants who qualify for licensure by examination by means of sections 12-265-108(1)(b) or (c), C.R.S., must have successfully completed study in colleges or universities approved by an organization nationally recognized by the United States Department of Education for regional institutional accrediting.
- B. The Board shall admit to examination for licensure as a nursing home administrator any applicant who has successfully completed one or more of the following within two years immediately preceding the date the application is received:

1. A Board approved Administrator-In-Training (AIT) program composed of at least one-thousand (1,000) hours, as set forth in Rule 1.12 below;
 2. A Bachelor's degree or higher degree in Public Health Administration or Health Administration;
 3. A Master's degree in Management or Business Administration;
 4. Any degree deemed appropriate by the Board including, but not limited to, the following:
 - a. A Bachelor's degree or higher degree in Health Care Management; or
 - b. A Bachelor's degree or higher degree in Nursing Home Administration.
 5. Health Services Executive (HSE) qualification.
- C. The Board shall admit to examination for licensure as a nursing home administrator any applicant who has successfully completed an Associate's degree or higher degree in a health care-related field or a Bachelor's degree in Business or Public Administration AND a minimum of one year of experience in administration in a nursing home or hospital. The experience must be within two years immediately preceding the date the application. A registered nurse who is a graduate of a three-year diploma program meets the associate degree requirement.
- D. Degrees in a health-care related field shall include, but not be limited to, the following:
1. Nursing;
 2. Social Services;
 3. Sociology;
 4. Physical Therapy; or
 5. Occupational Therapy.
- E. Degrees the Board does not deem to be in a health-care related field shall include, but not be limited to, engineering, language arts, communication, political science, or theology.
- F. Experience in administration shall require:
1. Day-to-day, on-site responsibility for supervising, directing, managing, monitoring, or exercising reasonable control over subordinates for one year;
 2. The coordination of interdisciplinary patient care and services;
 3. Planning, organizing, and controlling events toward a goal (examples include involvement in finance, budget, and policy-making decisions); and
 4. Experience gained in a hospital setting shall be in one or more of the following areas:
 - a. Long-term acute care hospital;
 - b. Transitional care unit;
 - c. "Swing bed" unit; or

d. Behavioral health hospital.

- G. For purposes of section 12-265-108(1)(c), C.R.S., experience in administration shall not include housekeeping, laundry, food service, activities, medical records, bookkeeping, or clerical responsibilities.

1.8 LICENSURE BY ENDORSEMENT

This Rule is promulgated pursuant to sections 12-20-202(3), 12-20-204, 12-265-107(1)(a), and 12-265-112, C.R.S.

- A. To be considered for licensure by endorsement pursuant to the Occupational Credential Portability Program under sections 12-20-202(3) and 12-265-112, C.R.S., an applicant must submit a completed application form, all supporting documentation, and the appropriate fee.
- B. Applicant must possess a current, active and unrestricted license in good standing or hold a military occupational specialty, as defined in section 24-4-201, C.R.S., and engaged in active practice for at least one year immediately preceding the date the application is received.
- C. The applicant has passed the national examination administered by the nationally and Board recognized testing entity for nursing home administrators and has passed any other examination in the state or territory in which he or she is licensed to practice nursing home administration.
- D. The applicant will schedule and pass the Colorado state exam within one year of the date they are eligible to sit for the exam.

1.9 TEMPORARY LICENSURE

This Rule is promulgated pursuant to sections 12-20-204, 12-265-107(1)(a), and 12-265-110, C.R.S.

- A. Applicants must have submitted an application for licensure as a nursing home administrator prior to consideration for any temporary license.
 - 1. If the applicant is the current Director of Nursing at the facility and is eligible for a temporary license for an emergency situation as stated below in section (B), then the applicant is not required to submit an application for licensure prior to consideration for any temporary license.
- B. Temporary License for Emergency Situations
 - 1. A temporary license, not to exceed ninety days, may be issued to a qualified applicant in the case of death of the administrator, termination of the administrator, resignation of the administrator, or other similar emergent circumstances. Promotion or transfer made at the discretion of the ownership, management, or facility governing board does not qualify as an emergency.
 - 2. The Program Director is authorized to issue one ninety day emergency temporary license, and one ninety day extension per occurrence.
 - 3. A qualified applicant for an emergency temporary license is one who meets the requirements for nursing home administrator licensing and may include the following:
 - a. An individual who has successfully completed the Administrator-in-Training (AIT) program; or

- b. An individual who qualifies for licensure by exam, endorsement, or reinstatement and has made application for licensure.
 - 4. An applicant for an emergency temporary license must submit an application and payment of the appropriate fee. A representative of the facility must attest to the nature of the emergency.
 - 5. The applicant cannot practice in the capacity of a nursing home administrator until the Board has approved the application for a temporary license.
- C. Temporary License for a Hospital Administrator
 - 1. A temporary license, not to exceed twelve months, shall be issued to a qualified applicant in the case of death of the administrator, termination of the administrator, resignation of the administrator, or other similar emergent circumstances. Promotion or transfer made at the discretion of the ownership, management, or facility governing board does not qualify as an emergency.
 - a. The Program Director is authorized to issue one six-month temporary license, and one ninety day extension per occurrence.
 - b. Any request for a second and final ninety day extension requires Board review and approval.
 - 2. A qualified applicant is one who meets the requirements for nursing home administrator licensing and may include the following:
 - a. An individual who has successfully completed the Administrator-in-Training (AIT) program; or
 - b. An individual who qualifies for licensure by exam, endorsement, or reinstatement and has made application for licensure.
 - 3. An applicant for a temporary license must submit:
 - a. An application and payment of the appropriate fee; and
 - b. A letter from the general hospital board of directors or similar authority verifying that the applicant is currently employed as the hospital administrator and that the hospital needs the applicant to serve as the nursing home administrator for one of the reasons set forth in section (B) above.
 - 4. The applicant cannot practice in the capacity of a nursing home administrator until the Board has approved the application for a temporary license.
 - 5. A temporary license issued to a hospital administrator under this section (C) shall be void at such time the license holder is no longer employed by the general hospital.

1.10 RENEWAL OF LICENSURE

This Rule is promulgated pursuant to sections 12-20-202, 12-20-204, 12-265-107(1)(a), and 12-265-110, C.R.S.

- A. A licensee is required to timely renew annually and submit the applicable fee. If a licensee fails to timely renew, the license shall expire.

- B. A licensee shall have a sixty day grace period after the expiration of the license to renew without the imposition of a disciplinary sanction for practicing on an expired license. A delinquency fee will be charged for late renewal during this grace period. A licensee who does not renew his or her license within the sixty day grace period shall be treated as having an expired license and therefore, ineligible to practice until such license is reinstated.

1.11 REINSTATEMENT OF AN EXPIRED LICENSE

This Rule is promulgated pursuant to sections 12-20-202, 12-20-204 and 12-265-107(1)(a), C.R.S.

- A. To be considered for licensure reinstatement, the applicant must submit a completed application for reinstatement with a reinstatement fee. The Board may require additional information to determine the applicant's ability and competence to practice with reasonable skill and safety based on the information provided by the applicant.
- B. The Board may deny an applicant for reinstatement upon a finding that the applicant has violated any provisions of the statute and regulations.
- C. If the license has been expired for more than two years, the applicant must submit a current National Practitioner Data Bank - Healthcare Integrity and Protection Data Bank (NPDB-HIPDB) report.
- D. If the license has expired for more than two years, the applicant shall satisfactorily demonstrate to the Board that he or she is competent to practice as a nursing home administrator. The Board shall require the following as a demonstration of competency to practice:
1. Documentation of active practice in another state for the two years immediately preceding the filing of the reinstatement application;
 2. Practice under a probationary or otherwise restricted license for a specified period of time;
 3. Successful completion of courses approved by the Board; or
 4. Any other professional standard or measure of continued competency as determined by the Board, including successful completion of the state and/or national examination.
- E. An applicant may petition the Board for reinstatement with a waiver of the competency requirements in this Rule, upon demonstration of hardship. The Board, at its discretion, may grant such waiver and reinstatement so long as the public is protected.

1.12 ADMINISTRATOR-IN-TRAINING (AIT) PROGRAM

This Rule is promulgated pursuant to sections 12-20-204, 12-265-107(1)(a), and 12-265-109, C.R.S.

- A. The AIT program is the way in which a person seeking licensure as a nursing home administrator may obtain practical training and experience in nursing home administration under the direct supervision of a licensed nursing home administrator (preceptor) who is in full-time practice at the facility.
- B. Education and experience criteria for admission into the AIT program.
1. The applicant must have two years college level study in areas relating to health care or two years of experience in nursing home administration or comparable health care management for each year of required education.

- a. College level study
 - (1) Study must be successfully completed in a college or university approved by an organization nationally recognized by the United States Department of Education for regional institutional accrediting.
 - (2) The types of courses which will be considered "relating to health care" for entry into the ALT program include, but are not limited to:
 - (a) Nursing;
 - (b) Social services (if related to health care);
 - (c) Recreational therapy (not physical education);
 - (d) Dietary;
 - (e) Psychology;
 - (f) Sociology;
 - (g) Physiology;
 - (h) Anatomy;
 - (i) Biology;
 - (j) Speech;
 - (k) Health care management; or
 - (l) Administration.
- b. Experience
 - (1) Experience must be obtained in nursing home administration or comparable health care management.
 - (2) Experience in comparable health care management may include, but is not limited to:
 - (a) Direct health care, which includes nursing, therapies, clinical social services, and activities;
 - (b) Ancillary services, which includes business office, medical records, environmental services, human resources, and marketing; or
 - (c) Planning, organizing and controlling events toward a goal such as involvement in finance, budget, and policy making decisions.
- c. The Board may approve substitution of two years of Board approved experience for one year of education, or may approve proportional experience for various amounts of education.

- d. Applicants will not be required to possess experience in administration as defined in Rule 1.7(E) in order to apply for the AIT program. This experience will be gained through participation in the program.
- e. The Board, on a case-by-case basis, may waive any portion of the college level study and/or experience specified in this Rule 1.12(B)(1) for an applicant submitting military training, experience, or education as part of the application for admission into the AIT program.

C. Applying for the AIT program

1. An applicant for the nursing home AIT program shall meet the statutory requirements of section 12-265-109, C.R.S., and shall submit the following:
 - a. A complete and accurate application form;
 - b. If applicable, transcripts sent directly to the Board;
 - c. Any requested additional or supplemental documentation;
 - d. An application for approval of a preceptor, signed by the applicant and the proposed preceptor. It shall be the responsibility of the AIT applicant to enter into an agreement with a Colorado licensed nursing home administrator who meets the qualifications to be a preceptor and who is willing to accept and supervise the AIT applicant in accordance with Board rules; and
 - e. A full AIT program plan for a period of 1,000 hours, with supporting documentation, not to exceed one year. The program plan must include a detailed timetable of planned learning experiences and a description of objectives. The program plan must include the methods of instruction, the hours and days of the week to accomplish the AIT program separate from other job duties, and a detailed description of the program of study addressing the following areas:
 - (1) Principles of administration including budgeting, accounting, records management, organization, personnel, and business management;
 - (2) Medical, sociological, psychological, environmental, quality assurance, and therapeutic aspects of resident care; and
 - (3) Federal, state, and local laws and regulations in relation to the administration of a nursing home.
2. The AIT shall work under the personal direction of the preceptor for the duration of the approved program. It is the responsibility of the preceptor and the AIT to ensure that the AIT is engaged in the AIT program in order for the hours to be considered.
3. The AIT program application must be approved in full by the Board prior to the applicant beginning the plan.
4. Before approving the applicant for the AIT program, the Board must be satisfied that the applicant does not have a substantial financial interest in the nursing home which is to be the site wherein the major portion of the training time will be served.

5. Once the applicant has successfully completed the AIT program, then a complete application for licensure by examination must be submitted. Please refer to Rule 1.7 for further information concerning that process.
6. Unless good cause is demonstrated to the Board, persons who have participated in an AIT program must successfully complete both the national and state examinations, and meet all other licensing requirements within one year after completion of the program.

D. Waivers

1. The Board may consider an applicant's request for waiver of any portion of the AIT period provided the applicant has had responsibility for performing all of the administrative or supervisory tasks in a nursing home or hospital in any of the following categories including, but not limited to:
 - a. Budgeting, accounting, records management, organization, personnel, and business management.
 - b. Medical, sociological, psychological, environmental, quality assurance, and therapeutic aspects of resident care.
2. Waiver up to, but no more than, 500 hours may be considered for an applicant who has had a minimum of one year of experience in administration, as defined in Rule 1.12(D)(4) below, in a nursing home in one of the categories set forth in Rule 1.12(D)(1) above.
3. In addition to the waivers set forth in Rule 1.12(D)(1) and (D)(2) above, the Board may consider a request for waiver of any portion of a state or the District of Columbia recognized AIT program upon submission by the applicant of documentation establishing that the applicant was accepted into such program and successfully completed the portion for which he or she is seeking waiver.
4. Waiver considerations
 - a. Experience in administration shall require:
 - (1) Day-to-day, on-site responsibility for supervising, directing, managing, monitoring, or exercising reasonable control over subordinates;
 - (2) The coordination of interdisciplinary patient care and services; and
 - (3) Planning, organizing, and controlling events toward a goal. Examples include involvement in finance, budget, and policy-making decisions.
 - b. Experience in administration shall not include housekeeping, laundry, food services, activities, medical records, bookkeeping, or clerical responsibilities.
 - c. Experience gained in a hospital setting shall be in one or more of the following areas:
 - (1) Long-term acute care hospital;
 - (2) Transitional care unit;
 - (3) "Swing bed" unit; or

(4) Behavioral health hospital.

5. If the applicant requests a waiver, he or she must submit a detailed plan outlining the requested areas of waiver as well as the areas of study to be completed. The detailed waiver plan must be submitted in addition to the full 1,000 hour program.
6. Once the Board reviews and takes action on a waiver request submitted at the time of the application, it will not consider any further waivers for that AIT.

E. Preceptors

1. A nursing home administrator is eligible to be a “preceptor” if he or she:
 - a. Has been licensed for no less than three years and has actively practiced nursing home administration for the preceding twenty-four months in the state of Colorado. At the Board’s discretion, it may reduce or waive the twenty-four month active practice requirement on a case-by-case basis. The Board may consider complaint, survey, and disciplinary history when reviewing preceptor applications.
 - b. Agrees to abide by the terms of the AIT agreement set forth by the Board for any AIT under their supervision.
2. No preceptor shall have more than one AIT under supervision at any one time unless pre-approval is obtained from the Board.
3. A preceptor shall remain fair and impartial during review of the AIT’s performance. No preceptor shall receive any financial remuneration from or on behalf of an AIT for services as a preceptor under the program. A preceptor shall not be related by blood or marriage to the AIT nor shall the preceptor have a personal financial interest in the licensure of the AIT. An AIT applicant in a rural or remote area may request a waiver of this requirement upon demonstration of hardship and that the preceptor can, and will, remain fair and impartial during the AIT program. Any such request for a waiver shall be in writing and shall be subject to the full discretion of the Board.
4. Each preceptor shall serve at the discretion of the Board.
5. All preceptors shall comply with the rules and regulations regarding the AIT program and may be disciplined for failure to do so.
6. All preceptors shall abide by the terms of any agreement entered into with the Board to act as a preceptor.
7. It is the responsibility of the preceptor to assist the AIT applicant in developing the detailed program plan. Such program may include classroom experience received while in the AIT program if Board approval is obtained.

F. Monitoring

1. Each AIT shall submit to the Board monthly progress reports on a form prescribed by the Board. The monthly progress reports submitted by the AIT shall be co-signed by the AIT and the preceptor and submitted with the Board no more than ten days following the end of the reporting period.
2. If an AIT fails to submit reports as prescribed above, the AIT may be deemed to have abandoned the AIT program.

3. Any falsification or misrepresentation contained in any report or document attesting to the facts, conditions, and activities of the AIT and submitted by the AIT, preceptor, or other licensees under the jurisdiction of the Board may be considered conduct which fails to meet generally accepted standards for nursing home administration practice and may subject the AIT, preceptor, or other licensees to discipline pursuant to section 12-265-113, C.R.S.
 4. The Board or its designee shall have the right to monitor, call for conference any party, or visit the facility during the course of the AIT program to assure compliance with these rules.
 5. The AIT and the preceptor shall be held responsible to adhere to Rule 1.15 during participation in any AIT program. Failure to do so may be grounds for denial of licensure and/or action on the license of the preceptor.
- G. Interruption, discontinuance, or disqualification
1. Termination of the AIT/preceptor agreement shall be reported by the AIT in writing to the Board within seven days of such termination.
 2. Discontinuance by the AIT of the program shall be reported by the AIT in writing to the Board within seven days after such discontinuance.
 3. The AIT program, in whole or in part, may be disqualified if the AIT violates any Board rule. Additionally, the Board may deny the AIT's application for licensure for violation of any Board rule.
 4. Acceptance into the AIT program in no way implies authority for the AIT to serve in the capacity of a nursing home administrator; such action by the AIT may disqualify the entire period of the AIT program.
 5. In the event the preceptor fails to provide the AIT an opportunity for adequate professional and occupational experience, the Board may disqualify the preceptor from further service in the AIT program.
 6. In the event that during the AIT program the training experience proves unsatisfactory, the Board may terminate or rearrange all or part of the program.

(AMENDED ON JULY 29, 2009 AND EFFECTIVE AUGUST 30, 2009; AMENDED ON JANUARY 27, 2010 AND EFFECTIVE MARCH 30, 2010; AMENDED ON MARCH 26, 2010 AND EFFECTIVE JULY 15, 2010; AMENDED ON MARCH 7, 2012 AND EFFECTIVE APRIL 30, 2012; AMENDED ON JULY 11, 2012 AND EFFECTIVE SEPTEMBER 1, 2012; AMENDED ON SEPTEMBER 5, 2012 AND EFFECTIVE OCTOBER 30, 2012; AMENDED NOVEMBER 2, 2012 AND EFFECTIVE DECEMBER 30, 2012; AMENDED ON JULY 17, 2013 AND EFFECTIVE AUGUST 30, 2013)

1.13 STANDARDS OF PRACTICE

This Rule is promulgated pursuant to sections 12-20-204 and 12-265-107(1)(a), C.R.S.

- A. Pursuant to section 12-265-107(2)(a), C.R.S., the following requirements must be met by any individual who receives and retains a license or a temporary license as a nursing home administrator. The licensee must:

1. Comply with all local, state, and federal laws and regulations concerning the operation or reimbursement of nursing homes and/or nursing home administrators. The licensed nursing home administrator shall cooperate with any investigation of these regulatory bodies.
2. Exercise reasonable supervision over the activities of nursing home personnel.
3. Maintain a clean and safe environment to ensure the health, safety, and welfare of residents and staff in the licensee's facility.
4. Ensure a quality of care and quality of life that is consistent with the health and safety of the residents in the licensee's facility. This includes, but is not limited to, the promotion of care, related medical and other services provided by the facility to assist each resident to attain or maintain the highest practicable mental, physical, and psychosocial well-being to the extent it is consistent with the resident's wishes.
5. Protect resident rights as required by state and federal laws including, but not limited to, the protection against abuse, neglect, and other mistreatment.
6. Participate with the ownership, management, or facility governing board to plan, implement, and evaluate written policies and procedures to promote facility systems to function properly and ensure compliance with law and regulation.
7. Communicate and problem solve regularly with the governing body, department heads, facility staff, and residents to allocate resources properly. The nursing home administrator shall also foster effective communication and problem solving between management, staff, residents, family, community, and all parties involved to ensure appropriate management and operation of the facility and to provide for residents' rights, health, safety, and welfare.
8. Plan, implement, and evaluate an integrated financial program for the facility which ensures compliance with applicable laws and regulations; quality of care and life; and appropriate and accurate billing for services.
9. Possess and maintain the competencies necessary to effectively perform his/her responsibilities as a nursing home administrator through the following list of competencies, which is for illustrative purposes only and is not intended to be exhaustive:
 - a. Exercising ethical and professional decision making and judgment;
 - b. Assuming leadership in the facility;
 - c. Exemplifying an administrative philosophy congruent with the mission and goals of the organization as well as generally accepted standards;
 - d. Planning, organizing, and directing those responsibilities delegated to the administrator by the ownership, management, or governing board of the facility and inherent in the role of an administrator;
 - e. Abiding by and keeping confidential resident information; and
 - f. Keeping current with trends and industry standards of practice.

10. Take appropriate steps to avoid discrimination on basis of race, color, sex, religion, age, national origin, disability, marital status, ancestry, sexual orientation, or any other factor that may be discriminatory or not related to bona fide requirements of quality care.
 11. Provide appropriate systems to account for and to protect residents' personal funds and property within the facility. It shall be a violation of this Rule to misappropriate resident or facility funds or property.
 12. Disclose to the ownership, management, or facility governing board or other authority as may be appropriate, any actual or potential circumstance concerning the nursing home administrator that might reasonably be thought to create a conflict of interest or have an adverse impact on the facility or its residents.
 13. Develop, participate, and monitor compliance of a functioning Quality Assurance Committee or its equivalent that evaluates and monitors the quality of resident care, residents' rights, and quality of life as required by Federal and State Regulations and generally accepted standards.
 14. Adhere to the generally accepted standards of practice of a Colorado licensed nursing home administrator including, but not limited to, the development and implementation of policies and procedures to ensure the health and safety of the residents.
- B. Any failure to meet these standards may result in disciplinary proceedings as set forth in section 12-265-113 and section 12-265-116, C.R.S.

(AMENDED ON JULY 29, 2009 AND EFFECTIVE ON AUGUST 30, 2009; AMENDED ON MARCH 7, 2012 AND EFFECTIVE APRIL 30, 2012; AMENDED ON JULY 11, 2012 AND EFFECTIVE SEPTEMBER 1, 2012; AMENDED ON JULY 17, 2013 AND EFFECTIVE AUGUST 30, 2013)

1.14 DECLARATORY ORDERS

This Rule is promulgated pursuant to sections 12-20-204, 12-265-107(1)(a), and 24-4-105(11), C.R.S.

- A. Any person may petition the Board for a declaratory order to terminate controversies or to remove uncertainties as to the applicability to the petitioner of any statutory provision or rule or order of the Board.
- B. The Board will determine, in its discretion and without notice to petitioner, whether to rule upon any such petition. If the Board determines that it will not rule upon such a petition, the Board shall promptly notify the petitioner of its action and state the reasons for such action.
- C. In determining whether to rule upon a petition filed pursuant to this Rule, the Board will consider the following matters, among others:
1. Whether a ruling on the petition will terminate a controversy or remove uncertainties as to the applicability to the petitioner of any statutory provision or rule or order of the Board;
 2. Whether the petition involves any subject, question, or issue which is the focus of a formal or informal matter or investigation currently pending before the Board or a court but not involving any petitioner;
 3. Whether the petition seeks a ruling on a moot or hypothetical question or will result in an advisory ruling or opinion; or

4. Whether the petitioner has some other adequate legal remedy, other than an action for declaratory relief pursuant to C.R.C.P. 57, which will terminate the controversy or remove any uncertainty as to the applicability to the petitioner of the statute, rule, or order in question.
- D. Any petition filed pursuant to this Rule shall set forth the following:
1. The name and address of the petitioner and whether the petitioner is licensed pursuant to the provisions of section 12-265-101, C.R.S., et seq., as amended;
 2. The statute, rule, or order to which the petition relates; and
 3. A concise statement of all of the facts necessary to show the nature of the controversy or uncertainty and the manner in which the statute, rule, or order in question applies or potentially applies to the petitioner.
- E. If the Board determines that it will rule on the petition, the following procedures apply:
1. The Board may rule upon the petition based solely upon the facts presented in the petition. In such a case, any ruling of the Board will apply only to the extent of the facts presented in the petition and any amendment to the petition.
 2. The Board may order the petitioner to file a written brief, memorandum, or statement of position.
 3. The Board may set the petition, upon due notice to the petitioner, for a non-evidentiary hearing.
 4. The Board may dispose of the petition on the sole basis of the matters set forth in the petition.
 5. The Board may request the petitioner submit additional facts in writing. In such event, such additional facts will be considered as an amendment to the petition. The Board may take administrative notice of the facts pursuant to the State Administrative Procedure Act, section 24-4-105(8), C.R.S., and may utilize its experience, technical competence, and specialized knowledge in the disposition of the petition.
 6. If the Board rules upon the petition without a hearing, it shall promptly notify the petitioner of its decision.
 7. The Board may, in its discretion, set the petition for hearing, upon due notice to the petitioner, for the purpose of obtaining additional facts or information or to determine the truth of any facts set forth in the petition or to hear oral argument on the petition.
 8. The notice to the petitioner setting such hearing shall set forth, to the extent known, the factual or other matters into which the Board intends to inquire.
 9. For the purpose of such a hearing, to the extent necessary, the petitioner shall have the burden of proving all of the facts stated in the petition, all of the facts necessary to show the nature of the controversy or uncertainty, the manner in which the statute, rule or order in question applies or potentially applies to the petitioner, and any other facts the petitioner desires the Board to consider.

- F. The parties to any proceeding pursuant to this Rule shall be the Board and the petitioner. Any other person may seek leave of the Board to intervene in such a proceeding, and leave to intervene will be granted at the sole discretion of the Board. A petition to intervene shall set forth the same matters as required by section (D) of this Rule. Any reference to a "petitioner" in this Rule also refers to any person who has been granted leave to intervene by the Board.

(AMENDED ON JULY 29, 2009 AND EFFECTIVE ON AUGUST 30, 2009; AMENDED MAY 26, 2010 AND EFFECTIVE ON JULY 15, 2010; AMENDED ON MARCH 7, 2012 AND EFFECTIVE APRIL 30, 2012; AMENDED ON JULY 11, 2012 AND EFFECTIVE SEPTEMBER 1, 2012; AMENDED ON JULY 17, 2013 AND EFFECTIVE AUGUST 30, 2013)

1.15 REPORTING CONVICTIONS, JUDGMENTS AND ADMINISTRATIVE PROCEEDINGS

- A. A licensee as defined in section 12-265-103(2), C.R.S., shall inform the Board, in a manner set forth by the Board, within thirty days of any of the following occurrences:
1. The conviction of the licensee of a felony under the laws of any state or of the United States or plea of guilty or nolo contendere to a felony, as described in section 12-265-113(1)(b), C.R.S.
 2. A disciplinary action imposed upon the licensee by another jurisdiction that registers or licenses nursing home administrators, as described in sections 12-265-113(1)(c) and (e), C.R.S., including, but not limited to, a citation, sanction, probation, civil penalty, or a denial, suspension, revocation, or modification of a license or registration whether it is imposed by consent decree, order, or other decision, for any cause other than failure to pay a license fee by the due date or failure to meet continuing professional education requirements;
 3. Revocation or suspension by another state board, municipality, federal or state agency of any health care related license, other than a license to practice nursing home administration as described in section 12-265-113(1)(c), C.R.S.; or
 4. Any judgment, award, or settlement of a civil action or arbitration proceeding in which the licensee was a party, if the action or proceeding included any allegation of gross negligence, violation of specific standards of practice, fraud, or misappropriation of funds in the practice as a nursing home administrator; provided, however, a licensee shall notify the Board only when such civil judgment, settlement, or arbitration award directly involves the practice of nursing home administration.
- B. The notice to the Board shall include the following information:
1. If the event is an action by a governmental agency (as described above), the name of the agency, its jurisdiction, the case name, and the docket, proceeding or case number by which the event is designated, and a copy of the consent decree, order, or decision.
 2. If the event is a felony conviction, the court, its jurisdiction, the case name, the case number, a description of the matter or a copy of the indictment or charges, and any plea or verdict entered by the court. The licensee shall also provide to the Board a copy of the imposition of sentence related to the felony conviction and the completion of all terms of the sentence with forty-five days of such action.
 3. If the event concerns a civil action or arbitration proceeding, the court or arbiter, the jurisdiction, the case name, the case number, a description of the matter or a copy of the complaint, and a copy of the verdict, the court or arbitration decision, or, if settled, the settlement agreement and court's order of dismissal.

- C. The licensee notifying the Board may submit a written statement with the notice to be included with the licensee records.

(AMENDED ON JULY 29, 2009 AND EFFECTIVE ON AUGUST 30, 2009; AMENDED ON MAY 26, 2010 AND EFFECTIVE JULY 15, 2010; AMENDED ON MARCH 7, 2012 AND EFFECTIVE APRIL 30, 2012; AMENDED ON JULY 11, 2012 AND EFFECTIVE SEPTEMBER 1, 2012; AMENDED ON JULY 17, 2013 AND EFFECTIVE AUGUST 30, 2013)

1.16 Concerning Health Care Provider Disclosures to Consumers about the Potential Effects of Receiving Emergency or Nonemergency Services from an Out-of-Network Provider

This rule is promulgated pursuant to sections 12-20-204, 12-30-112, and 12-265-107(1)(a), C.R.S., in consultation with the Commissioner of Insurance and the State Board of Health. The purpose of this rule is to establish requirements for health care providers to provide disclosures to covered persons who are utilizing a health benefit plan about the potential of balance billing when receiving post-stabilization services or covered non-emergency services from an out-of-network provider at an in-network facility. This rule applies to health care providers. Balance billing by a health care provider is only permitted when the criteria established in Colorado law, including but not limited to sections 12-30-112 and 12-30-113, C.R.S., are met.

- A. Definitions, for purposes of this rule, are as follows:

1. "Ancillary Services" as defined in section 12-30-112(1)(a), C.R.S.
2. "Balance Bill" and "Balance Billing" as defined in section 10-16-704(19)(c), C.R.S.
3. "Covered Non-emergency Services" means services that are not emergency services as defined in section 10-16-704(19)(e), C.R.S., are services covered by a covered person's health benefit plan, and are not ancillary services as defined in section 12-30-112(1)(a), C.R.S.
4. "Covered Person" as defined by section 10-16-102(15), C.R.S.
5. "Health Benefit Plan" as defined by section 10-16-102(32), C.R.S.
6. "Health Care Provider" means "provider," as defined in section 10-16-102(56), C.R.S.
7. "In-Network Facility" means a facility, either within or outside of Colorado, that, under a contract with a carrier or with its contractor or subcontractor, has agreed to provide health-care services to covered persons with an expectation of receiving payment, other than coinsurance, copayments, or deductibles, directly or indirectly, from the carrier.
8. "Out-of-Network Provider" means a Health Care Provider who is not a "Participating Provider."
9. "Participating Provider" as defined in section 10-16-102(46), C.R.S.
10. "Post-Stabilization Services" means covered services related to an emergency medical condition, as defined in section 10-16-704(19)(d), C.R.S., that are provided once the criteria set forth in section 10-16-704(19)(e)(III) are met.

- B. Disclosure requirements.
 - 1. An Out-of-Network Provider may balance bill a Covered Person for (a) Post-Stabilization Services in accordance with section 10-16-704, C.R.S., and (b) Covered Non-Emergency Services provided in an In-Network Facility that are not Ancillary Services, but only if the Out-of-Network Provider meets the requirements set forth in section 12-30-112(3.5), C.R.S.
 - 2. If a Covered Person may incur a claim for Post-Stabilization Services or Covered Non-Emergency Services from an Out-of-Network Provider, the Out-of-Network Provider shall complete and provide the notice contained in Appendix "A" to these rules or a similar disclosure which complies with the requirements set forth in section 12-30-112(3.5), C.R.S.
 - 3. Such notice must be provided in the 15 most common languages in Colorado, which, for purposes of this regulation, are English, Spanish, Vietnamese, Chinese, Korean, Russian, Amharic, Arabic, German, French, Nepali, Tagalog, Japanese, Cushite, Persian.
- C. If applicable and in addition to their responsibilities under this Rule, Health Care Providers shall also comply with the "No Surprises Act," 42 U.S.C.A. § 300gg-111, Pub.L 116-260, as amended.
- D. Noncompliance with this Rule may result in the imposition of any of discipline made available by section 12-265-113(1)(d), C.R.S.

APPENDIX A

BALANCE BILLING NOTICE

PATIENT RIGHTS INFORMATION

Check the appropriate box:

- ☐ Your provider is proposing to use an out-of-network care provider in delivering your health care service(s). This facility is in-network with your insurance but there may be care providers involved in your care that are out-of-network.
- ☐ Your provider is proposing to deliver post-stabilization care at an out-of-network facility. You have received emergency services at the out-of-network facility and are now stabilized, but you may require additional health care services.

You are not required to consent to receive these services from the out-of-network care provider or continue to receive post-stabilization care at an out-of-network facility. If you choose to proceed with the proposed out-of-network care provider or facility you may be billed for costs detailed in the Good Faith Estimate below. The additional costs you pay may not accrue toward insurance cost sharing or deductibles.

You may choose to use an in-network provider from the list below or you may choose to transfer your care to an in-network facility for post-stabilization services. If you choose to proceed with an in-network provider or transfer to an in-network facility, the cost will not exceed the amount allowed by your insurance plan.

You chose to receive this Notice ☐ electronically or ☐ in paper form.

This notice must have been provided to you, either in paper or electronically, per your preference within the following timeframes:

1. At least seventy-two hours in advance of the date of services, if the appointment was scheduled at least seventy-two hours in advance;
2. At least three hours before the scheduled appointment, if the appointment was made less than seventy-two hours in advance.

This is not a contract for services. Your provider is required to retain this form for seven years.

This form must be available to you in the 15 languages most common to the geographic region where your provider is located, which include English, Spanish, Vietnamese, Chinese, Korean, Russian, Amharic, Arabic, German, French, Nepali, Tagalog, Japanese, Cushite, and Persian.

BILLING ADVISEMENT

(choose applicable billing scenario)

- ☐ Out-of-Network Provider at In-Network Facility

Your provider is proposing to use an out-of-network care provider in delivering your service(s). That out-of-network provider is/are:

[PROVIDER NAME]

Description of service(s) to be provided by an in-network facility by an out-of-network provider:

[SERVICE]

You scheduled the service(s) on [DATE] at [TIME]. You are planning to receive the service(s) stated above on [DATE] at [TIME]

Do you need prior authorization from your insurance company for the service(s) provided at this facility?

[Y / N]

Good Faith Estimate for the total cost of the service(s) to you, the patient:

[\$]

Does this facility employ in-network care providers who provide the service(s) detailed above?

[Y / N]

If Yes, the in-network care provider(s) who provide the service(s) are:

[PROVIDER NAME]

NOTE: If there is no in-network provider to provide the service(s) at this in-network facility you cannot be balanced billed for the services provided by the out-of-network provider. **OR**

☐ Post-Stabilization Services

Your provider is proposing to deliver post-stabilization care at an out-of-network facility. That out-of-network facility is and/or the provider(s) is/are:

[FACILITY/PROVIDER NAME]

Description of post-stabilization service(s) to be provided by an out-of-network facility or provider:

[SERVICE]

You scheduled the service(s) on [DATE] at [TIME]. You are planning to receive the service(s) stated above on [DATE] at [TIME].

Good Faith Estimate for the total cost of the service(s) to you, the patient:

[\$]

I [PATIENT NAME] received this form at [TIME] on [DATE].

SIGNATURE OF PATIENT

[TIME] and [DATE]

Editor's Notes

History

Rules 1-6 eff. 07/30/2008.
Rules 1-7 eff. 10/30/2008.
Entire rule eff. 08/30/2009
Entire rule eff. 03/30/2010.
Entire rule eff. 07/15/2010.
Rule 2.B.1 eff. 07/01/2011.
Entire rule eff. 04/30/2012.
Entire rule eff. 09/01/2012.
Rule III.E.1.a eff. 10/30/2012.
Rules II.A.5.b-c, II.C.2.c.ii, II.C.3, II.E.1, III.C.1, III.D.3 eff. 12/30/2012.
Entire rule eff. 09/14/2013.
Rules 1.3.B.1.e, 1.3.D.1, 1.3.D.4.c eff. 05/15/2020.
Rules 1.1 Q, 1.2 B eff. 01/14/2021.
Entire rule eff. 07/15/2021.
Rule 1.4 2 eff. 11/14/2021.
Rules 1.4 B-C, 1.6, 1.8, 1.9 A.1, 1.9 C.3.b, 1.9 C.5, 1.11 eff. 12/30/2021.
Rules 1.17, 1.18 emer. rules eff. 09/14/2022.
Rules 1.8 B, 1.16, 1.17, 1.18, Appendix A eff. 10/30/2022.
Rule 1.15 A.1 eff. 12/30/2023. Rules 1.17, 1.18 repealed eff. 12/30/2023.
Rules 1.7 B-G, 1.8 B-D eff. 05/30/2025.
Rule 1.16, Appendix A eff. 09/14/2025.